

MRI Pelvis rectum**Clinical information:****Findings****Primary tumour**

The primary tumour is demonstrated as an Annular Mucinous mass with a nodular infiltrating border.

The distal edge of the luminal tumour arises at a height of 35 mm from anal verge:

The distal edge of the tumour lies 22 mm below the top of the puborectalis sling

The tumour extends craniocaudally over a distance of 69mm

The distal edge of tumour lies entirely below the peritoneal reflection

Invading edge of tumour extends circumferentially

Tumour extends through the muscularis propria:

Extramural spread is 5.3 mm

Maximum mural and extramural thickness (maximum thickness of tumour) = 20 mm

mrT stage: T4 visceral

Tumour is present at the distal levator level:

Tumour extends into the external sphincter: intersphincteric plane/mesorectal plane is unsafe therefore an extralevator APE. is indicated for radial clearance.

Mesorectal lymph node assessment - Only benign reactive and no suspicious nodes shown N0

Extramural venous invasion:Evidence: Slight expansion of veins by tumour/ Clear and definite irregular expansion of vein (=mrEMVI POSITIVE), Large vein invasion is present.

mrEMVI positive: Venous invasion is affecting: the Inferior rectal or branches

CRM

The closest circumferential resection margin is at 3 o'clock

The closest CRM is from Extramural venous invasion

Minimum tumour distance to mesorectal fascia: 0 mm TME plane mrCRM involved, 1 mm or less (unsafe)

Peritoneal deposits: No evidence **Pelvic side wall lymph nodes:** None

Summary: yMRI Overall stage: ymrT4 N0 Mx, TME plane CRM involved, EMVI positive PSW negative
comparison with baseline pre treatment scans would be helpful to determine the degree of response

Examination: :MRI Pelvis rectum

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